Nigeria Centre for Disease Control and Prevention
Protecting the health of Nigerians

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Lassa Fever Situation Report

Epi Week 29: 13th – 19th July 2026

Key Points

Table 1: Summary of the current week (29), cumulative Epi week 1-29, 2026 and comparison with the previous year (2025)

Reporting Period	Suspected cases	Confirmed cases	Probable cases	Deaths (Confirmed cases)	Case Fatality Rate (CFR)	States and LGAs affected (Confirmed cases)
Current week (week 29)	173	20	0	2	10.0%	State(s):3 LGA(s):6
2026 Cumulative (week 29)	6727	983	6	231	23.5%	State(s):23 LGA(s): 114
2025 Cumulative (week 29)	6640	822	7	155	18.9%	State(s):21 LGA(s): 105

Highlights

- In week 29, the number of new confirmed cases decreased from 25 in epi week 28 of 2026 to 20. These were reported in Ondo, Bauchi and Edo States (Table 3).
- Cumulatively, 231 deaths have been reported with a Case Fatality Rate (CFR) of 23.5% which is higher than the CFR for the same period in 2025 (18.9%).
- In total for 2026, 23 States have recorded at least one confirmed case across 114 Local Government Areas (Figures 2 and 3).
- Eighty-six percent (86%) of all confirmed Lassa fever cases were reported from 5 states (Ondo, Bauchi, Taraba, Edo and Benue) while fourteen percent (14%) were reported from 18 states with confirmed Lassa fever cases. Of the 86% confirmed cases, Ondo reported 32%, Bauchi 25%, Taraba 13%, Edo 10% and Benue 6%.
- The predominant age group affected is 21-30 years (Range: 1 to 93 years, Median Age: 30 years). The male-to-female ratio for confirmed cases is 1:0.9 (Figure 4).
- The number of suspected and confirmed cases increased compared to that reported for the same period in 2025.
- No new healthcare worker was affected in the reporting week 29.
- National Lassa fever multisectoral multi-partner Technical Working Group (TWG) continues supporting the coordination of activities at all levels.

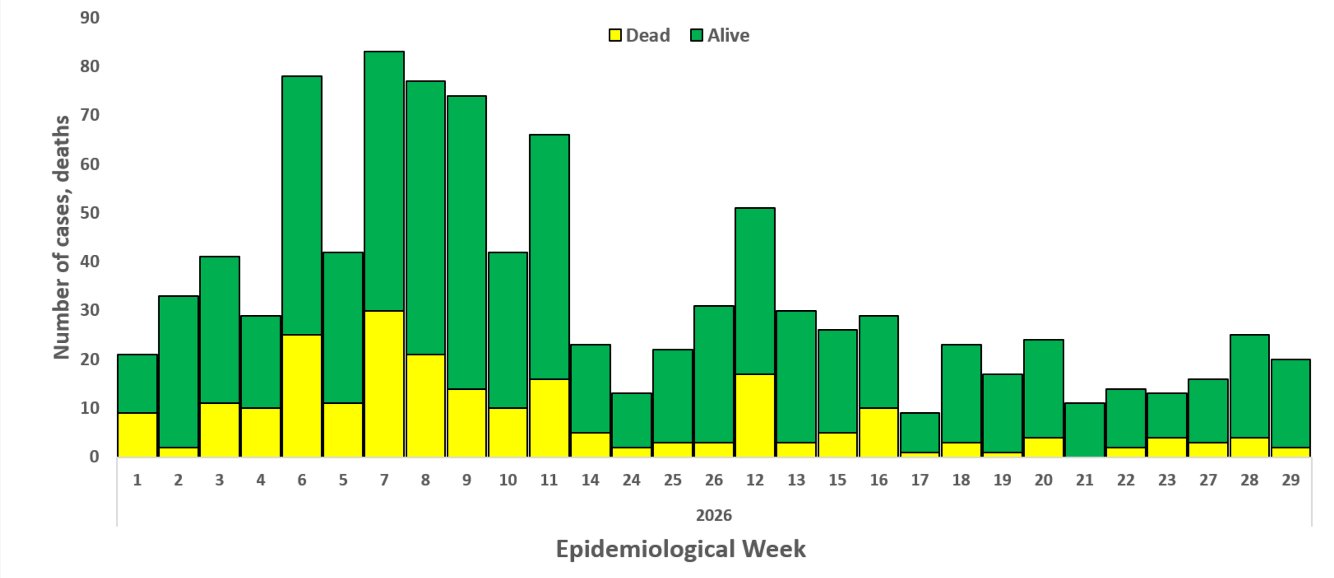

Figure 1. Confirmed Lassa Fever Cases in Nigeria Epidemiological Week 29, 2026

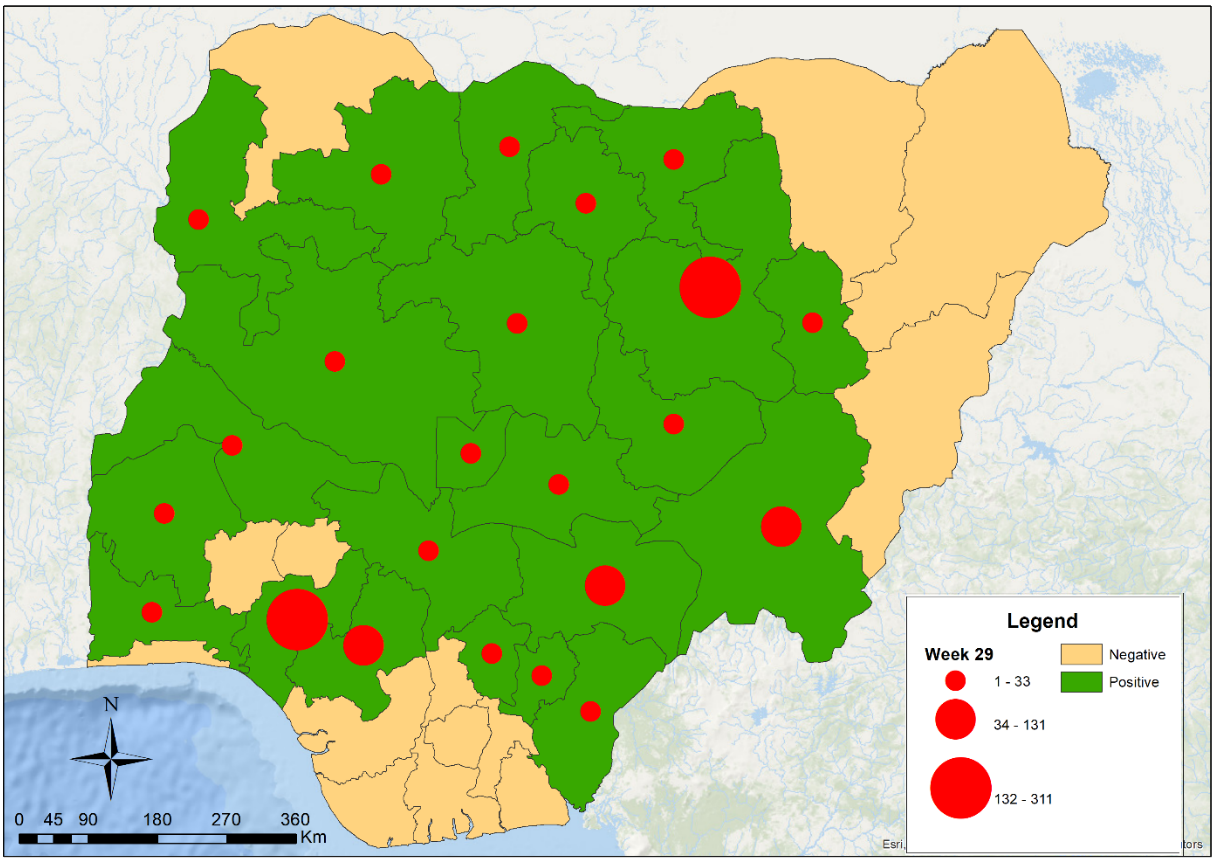

Figure 2. Confirmed Lassa fever cases by States in Nigeria, week 29, 2026

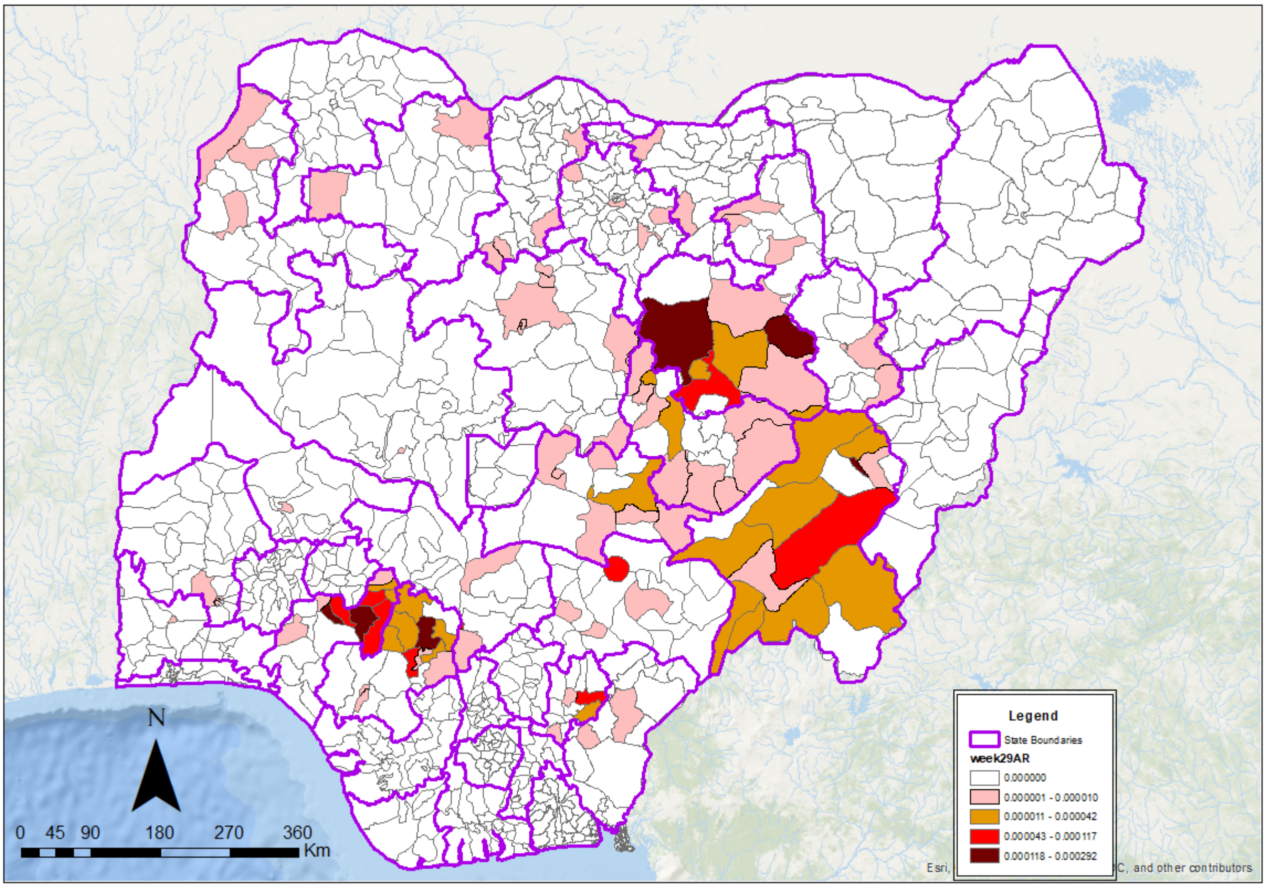

Figure 3. Confirmed Lassa fever attack rate per 100,000 population for LGAs in Nigeria, week 29 2026

Table 2: Key indicators for the current week in 2026 and trend compared to the previous week, Nigeria

Symptomatic contacts	Number for current week	Trend from previous week	Cumulative number for 2026
Probable cases	0	←→	**6**
Health Care Worker affected	0	↓	**52**
Cases managed at the treatment centres	18	↓	**752**
Contact tracing			
Cumulative contact listed	19	↓	**1897**
Contacts under follow up	118	↑	**118**
Contacts completed follow up	16	↑	**1776**
Symptomatic contacts	0	←→	**109**
Positive contacts	0	←→	**49**
Contacts lost to follow up	0	←→	**3**

Key

↑ Increase
↓ Decrease
↔ No difference

Table 3. Weekly and Cumulative number of suspected and confirmed cases for 2026

	States	Current week: (Week 29)						Cumulative (Week 1 - 29)				
		Cases					Deaths	Cases				Deaths
		Suspected	Confirmed	Trend	Probable	HCW*	(Confirmed Cases)	Suspected	Confirmed	Probable	HCW*	(Confirmed Cases)
1	Ondo	98	17	▲			2	1791	311		11	71
2	Bauchi	15	2	▼				1053	247	1	5	32
3	Taraba							364	131		3	43
4	Edo	45	1	▼				1426	94			21
5	Benue	4						416	58	2	15	14
6	Plateau							167	33	3	2	11
7	Ebonyi	4						277	24		3	12
8	Nasarawa							267	17		7	3
9	Kogi	5		▼				56	14			7
10	Kaduna							86	13			3
11	Kano							101	8		3	1
12	Gombe							56	7			4
13	Katsina							19	5			4
14	Oyo							119	5		3	1
15	Fct							52	3			
16	Kebbi							9	3			1
17	Zamfara							25	2			
18	Cross River			▼				29	2			1
19	Jigawa							49	2			2
20	Niger							12	1			
21	Kwara							15	1			
22	Ogun							25	1			
23	Enugu							60	1			
24	Akwa Ibom							2				
25	Ekiti	1						49				
26	Imo							8				
27	Sokoto							1				
28	Yobe							9				
29	Delta							35				
30	Rivers							9				
31	Anambra	1						14				
32	Osun							17				
33	Bayelsa							8				
34	Abia							9				
35	Borno							14				
36	Lagos							74				
37	Adamawa							4				
	Total	**173**	**20**	▼			**2**	**6727**	**983**	**6**	**52**	**231**

Key	
▼	Decrease
▲	Increase

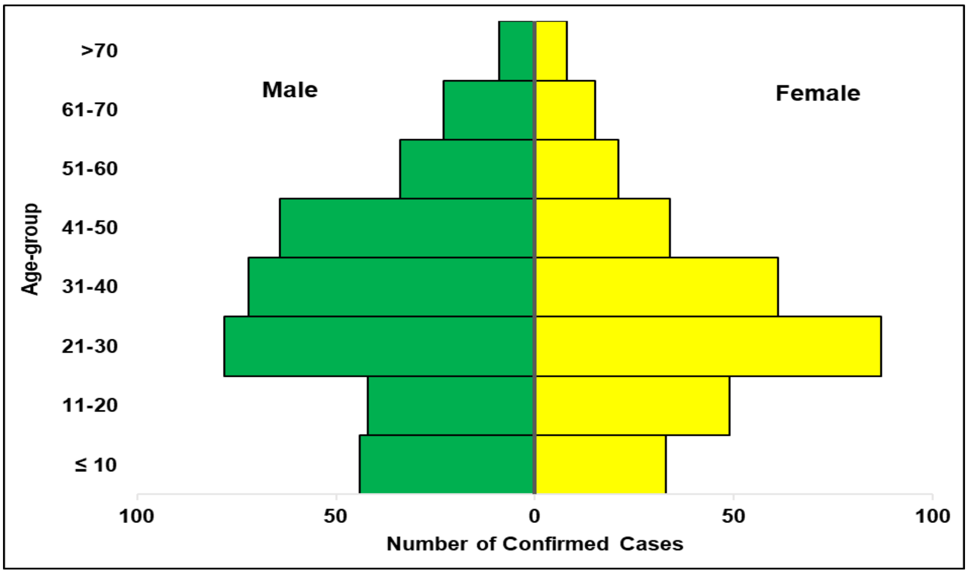

Figure 4. Age and sex pyramid showing the number of confirmed Lassa fever cases for 2026

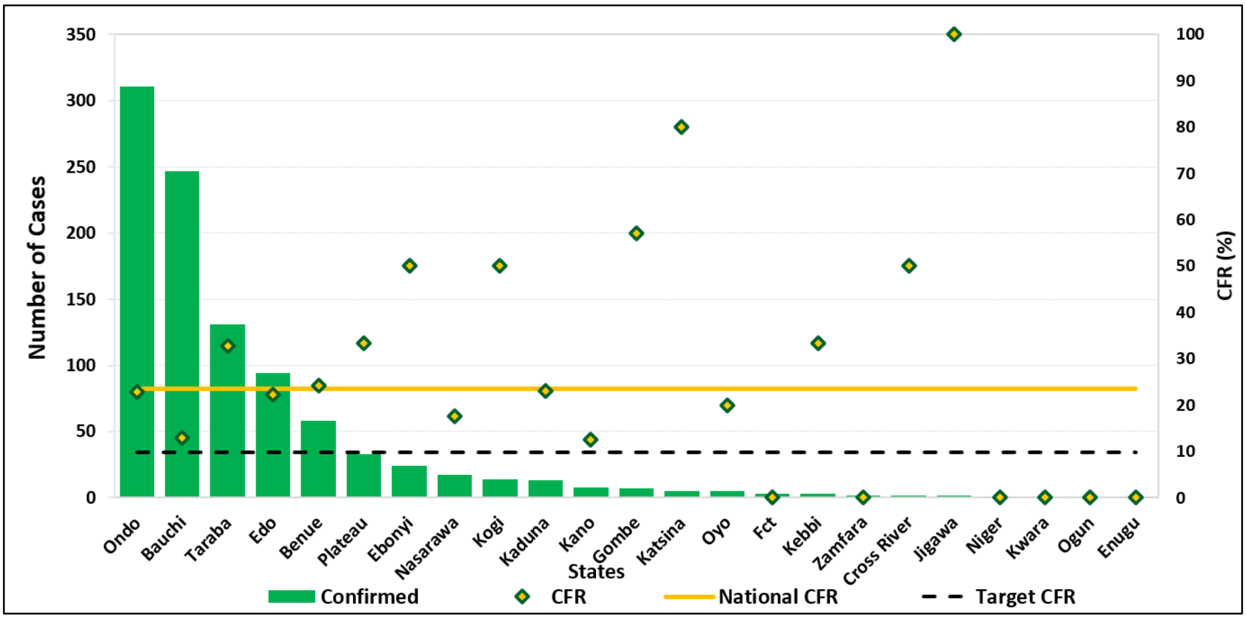

Figure 5: Number of confirmed cases with Case Fatality Rate (CFR) by state week 29, 2026

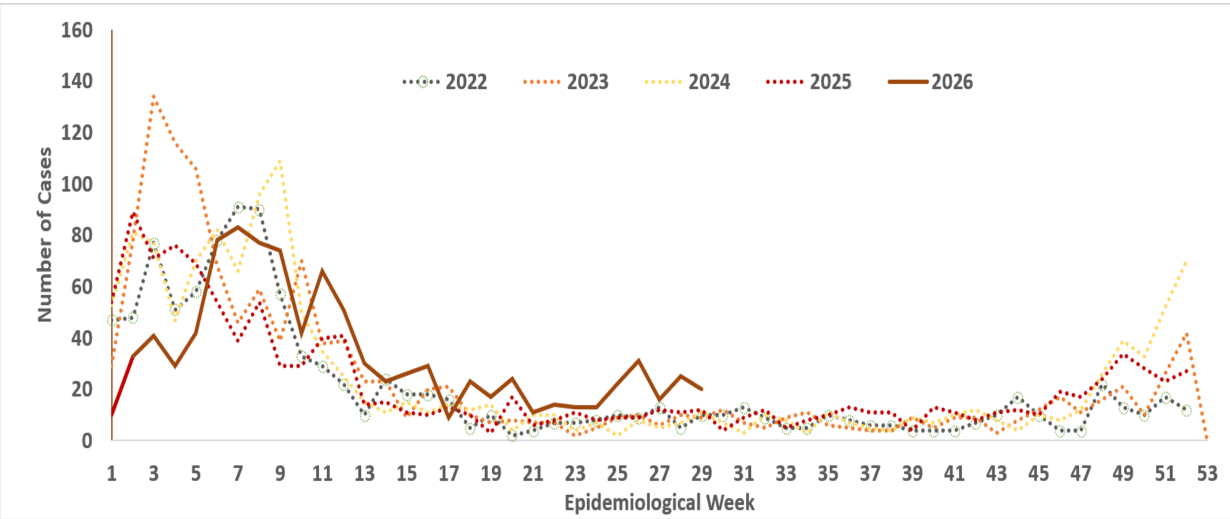

Figure 6: Trend of confirmed cases by epidemiological week, 2022– 2026, Nigeria

Response activities

- Supported Bauchi State and Plateau State Case Management training of HCWs in collaboration with the State Ministry of Health and FHI 360
- De-activation of the national Lassa fever Incident Management System to alert mode with continuous support of coordination efforts at all levels through the National Lassa fever TWG.
- Held a two-day webinar to advocate for the “Protection of the Public Health Workforce” in collaboration with APIN
- Held community engagement and capacity building workshop of surveillance actors for early referral of Lassa fever cases, with the support of Integrate consortium and ISTH in Edo state
- Trained frontline healthcare workers with the support of Africa CDC, WHO, MSF & FHI 360
- A dynamic risk assessment for the ongoing Lassa fever outbreak was done in collaboration with stakeholders
- Held a review of the National Case Management Guidelines (Workshop I) with the support of WHO
- Conducted workshop to strengthen State IPC programmes for Akwa-Ibom, Bauchi, and Lagos States
- Held the scheduled capacity building for 62 IPC focal persons across Lassa Fever ring facilities in Ebonyi and Ondo States, focusing on screening, triage, and prevention, with support from WHO.
- Incident Management System of the Public Health Emergency Operations Centre activated in Oyo State
- Risk Communication and Community engagement with religious and traditional leaders in Edo State and Ondo state
- IPC training for Health Care Workers from 5 Lassa fever endemic LGAs in Ondo state with the support of ALIMA (The Alliance for International Medical Action)
- Sensitization of farmers and market women in collaboration with National Orientation Agency in Ondo state
- Developed a 30-day Healthcare Worker Protection Plan for Lassa fever to curb infections among frontline healthcare workers in endemic states with the support of WHO, and US CDC
- Led a high-level advocacy to Benue State with the support of WHO, UNICEF and MSF
- Released a joint advisory for Medical Doctors and healthcare workers to bolster ongoing efforts for Lassa fever control and management
- Held a meeting with the Nigerian Medical Association (NMA) to strengthen collaborative efforts
- Piloted a targeted IPC Ring Strategy intervention in Benue State with the support of WHO
- Supporting high burden states with active case search and contact tracing in collaboration with the Research Triangle Institute (RTI) International through US CDC funding
- Supported the integrated case management training in Taraba state with the support of WHO
- Disseminated of updated IPC guidelines, SOPs as reminders for HCWs adherence
- Prepositioned PPE and distributed to health facilities
- Activation of Incident Management System for Lassa fever in Benue, Plateau, Kebbi, Kano, Gombe states
- Held the inaugural Joint Clinical Fellows Meeting (LFCMF Cohorts I & II) with the support of Georgetown University and US CDC.
- Administered the RCCE need assessment questionnaire across the National Rapid Response Teams (NRRT) deployed states
- Deployment of NRRT across 7 high burden states for the outbreak with the support of the BHCPF
- Held a pre-deployment briefing to ensure teams were adequately prepared for outbreak containment in the field
- Conducted a high-level field mission to Bauchi State with the support of Médecins Sans Frontières (MSF)
- Pilot implementation of the Turn a State Orange (TASO) Programme in Enugu, Oyo, and the FCT in collaboration with DRASA
- Collaborated with Logistic pillar to facilitate the distribution and pre-positioning of PPE at facilities with active and previous healthcare worker infections
- Technical support from US CDC and Pro-Health International to investigate and mitigate healthcare worker infections
- The APIN Orange Network is strengthening the capacity of health facilities in conducting Hand Hygiene Audits and implementing hand hygiene improvement programmes
- Analysed samples across the Laboratory network for Lassa fever to guide prompt diagnosis and treatment
- Supporting the implementation of the Community-based One Health Participatory and Empowerment (COPE) Phase II collaboration with RKI

- Shared soft copies of Lassa Fever (LF) Social Behavioural Change (SBC) materials with State Health Promotion Officers (SHPOs) and other RCCE stakeholders
  Supply of Lassa fever IPC commodities & drugs to BSUTH treatment & isolation centre with support from WHO
- Engaged with all stakeholders across the national and subnational Ministries of environment to prevent and control Lassa fever outbreaks

Challenges

- Late presentation of cases leading to an increase in CFR
- Poor health-seeking behaviour due to the high cost of treatment and clinical management of Lassa fever
- Poor environmental sanitation conditions observed in high-burden communities
- Poor awareness observed in high-burden communities
- Healthcare workers infection

Recommendations

- **States-** Bolster efforts all-year-round for community engagements on prevention of Lassa fever
- **Healthcare Workers-** Maintain high suspicion for Lassa fever and initiate timely referral and treatment, and adhere to standard infection prevention and control procedures.
- **NCDC/Partners-** Strengthen state capacity to prevent, detect and respond timely to Lassa fever

Notes on this report

Data Source

Information for this disease was case-based data retrieved from the National Lassa Fever Technical Working Group.

Case definitions

- **Suspected case**: any individual presenting with one or more of the following: malaise, fever, headache, sore throat, cough, nausea, vomiting, diarrhoea, myalgia, chest pain, hearing loss and either a. History of contact with excreta or urine of rodents b. History of contact with a probable or confirmed Lassa fever case within a period of 21 days of onset of symptoms OR Any person with inexplicable bleeding/haemorrhage.
- **Confirmed case**: any suspected case with laboratory confirmation (positive IgM antibody, PCR or virus isolation)
- **Probable case**: any suspected case (see definition above) who died or absconded without collection of specimen for laboratory testing
- **Contact:** Anyone who has been exposed to an infected person, or to an infected person's secretions, excretions, or tissues within three weeks of last contact with a confirmed or probable case of Lassa fever

Calculations

- ***Case Fatality Rate (CFR) for this disease is reported for confirmed cases only***.

VIRAL HAEMORRHAGIC FEVER QUICK REFERENCE GUIDE

For social mobilization https://ncdc.gov.ng/themes/common/docs/vhfs/83_1517222929.pdf
For LGA Rapid Response Team https://ncdc.gov.ng/themes/common/docs/vhfs/82_1517222811.pdf
Healthcare worker laboratory https://ncdc.gov.ng/themes/common/docs/vhfs/81_1517222763.pdf
For healthcare workers https://ncdc.gov.ng/themes/common/docs/vhfs/80_1517222586.pdf
For community informants https://ncdc.gov.ng/themes/common/docs/vhfs/79_1517222512.pdf

NATIONAL GUIDELINES FOR LASSA FEVER CASE MANAGEMENT

https://ncdc.gov.ng/themes/common/docs/protocols/92_1547068532.pdf

VIRAL HAEMORRHAGIC FEVER AND RESPONSE PLAN

https://ncdc.gov.ng/themes/common/docs/protocols/24_1502192155.pdf

NATIONAL GUIDELINE FOR INFECTION, PREVENTION AND CONTROL FOR VIRAL HAEMORRHAGIC FEVER INFORMATION RESOURCE

https://ncdc.gov.ng/themes/common/docs/protocols/341_1707300274.pdf

ADVOCACY TOOLKIT

https://ncdc.gov.ng/themes/common/docs/protocols/359_1739532942.pdf

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